

Public Health Emergency Operations Center
« COUSP RDC »

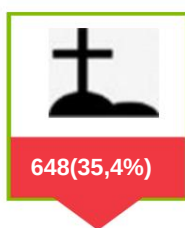
MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC
SitRep N°056/MVB_09/07/2026

Provinces affected (3)	EAST, NORTHWEST & SOUTHWEST
Affected Health Zones (37)	• Ituri (25/36): Aru, Aungba, Bambu, Bunia, Damascus, Drodro, Gety, Kilo, Commander, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa and Boga. • North Kivu (11/34) : Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Maseka, Pottery, Balconies et Musienene. • Sud-Kivu (1/34) : Miti-Murhesa
Date the report	July 9, 2026
Publication date	July 10, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



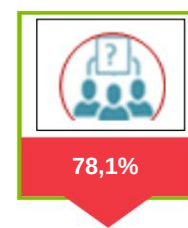
Patients in
isolation -
hospitalisation



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- Meeting with His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health, provincial authorities, members of the national coordination team, and technical and financial partners. Discussions focused primarily on the progress of the response and the situation in the provinces of Tshopo and Haut-Uélé.
- Over the past 24 hours, **38 new confirmed cases and 23 deaths (73.9% community transmission) have been recorded** in the provinces of Ituri (32 cases and 18 deaths) and North Kivu (6 cases and 5 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at **1,830 confirmed cases and 648 deaths**, representing an overall case fatality rate of **35.4%**.
- **Six (6)** deaths were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (4 at the Bunia CTE and 2 at the Mongbwalu CTE).
- **Five (5)** patients with Ebola virus disease in Ituri province (2 at the ISTM Ebola Treatment Center and 2 at the Ebola Treatment Center) Komanda) and North Kivu (1 at CTE Katwa) were declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km

from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus (EV) outbreak, which was then raging in North Kivu. The current outbreak, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.



Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 9, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of July 9, 2026.

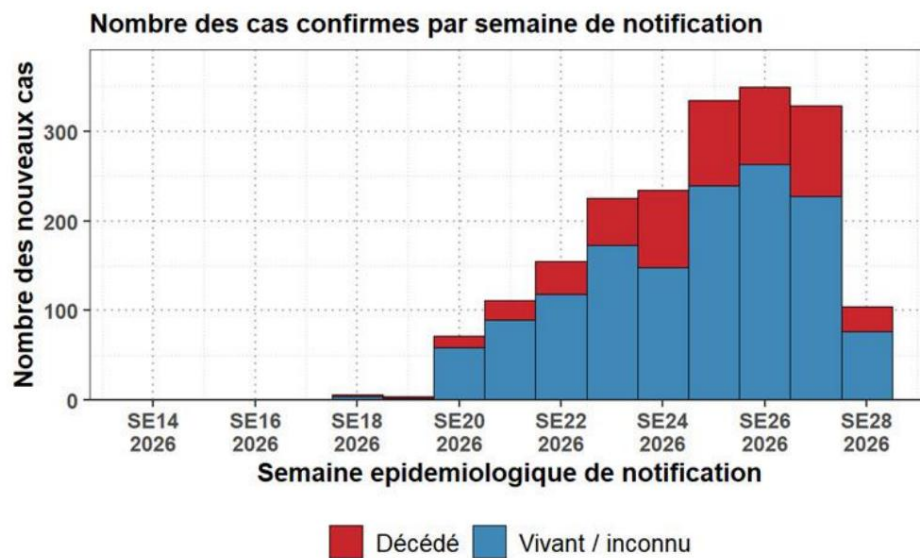
Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New cases confirmed (24h)
Ituri	1663	553*	33,3%	25 out of 36 (69.4%)	32
North Kivu	164	94	57,3%	11 out of 34 (32.4%)	6
South Kivu	3	1	33,3%	1 out of 34 (2.9%)	0
Total	1,830	648	35.4%	37 out of 104 (35.6%)	38

patients with EVD (previous confirmed cases) in ETD/CT.

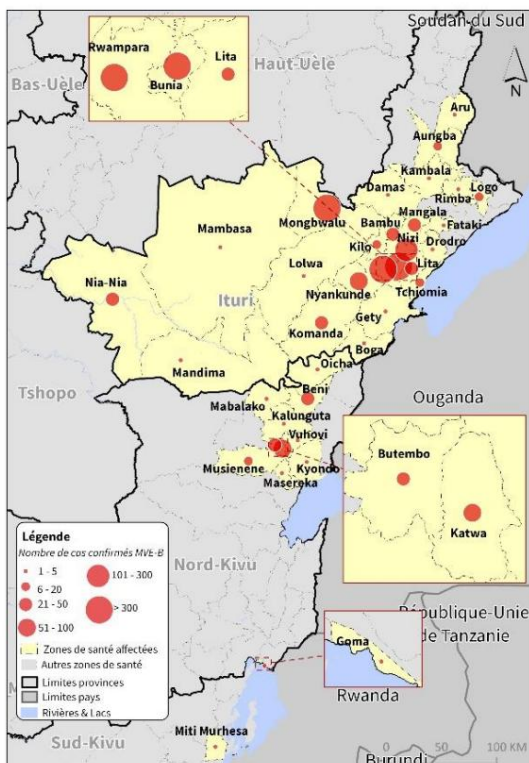
At the end of the day on July 9, 2026, no new health zones were affected. The number of affected health zones remains at 37/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission. As of July 7, 2026, South Kivu province officially passed the 42-day mark without a new confirmed case being reported, which corresponds to twice the maximum incubation period of the virus. This crucial milestone marks the end of the enhanced surveillance period and paves the way for the official declaration of the end of the epidemic in this province, subject to the maintenance of residual epidemiological vigilance.

3.2 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) confirmed continuous and increased community transmission at week 25. Compared to all weeks

where cases have been recorded since the start of the epidemic, weeks 25, 26 and 27 continue to see carryover growth, with more than 300 weekly cases.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 9, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 9, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1663	553	33,3%
Bunia	495	149	30,1%
Rwampara	373	83	22,3%
Mongbwalu	327	170	52,0%
Legumes	97	22	22,7%
Nizi	107	36	33,6%
Liter	42	17	40,5%
Team	25	8	32,0%
Bamboo	29	10	34,5%
Kilo	9	1	11,1%
Mangala	43	23	53,5%
Tchomia	17	4	23,5%
Ladies	5	0	0,0%
Aungba	6	2	33,3%
Jungle	4	0	0,0%
Understand	3		33,3%
Washing	5	1	80,0%
Logo	7	4	0,0%
Drodoro	3	0	100,0%
Tide		3	0,0%
Flounder	1	0	100,0%
Nanny	2	2	37,1%
Fireworks			25,0%
Darkness			40,0%
Fight			100,0%
have			100,0%
Other areas not yet identified	35 4 5 1 1 17	13 1 2 1 1 0	0,0%
NORTH KIVU	164	94	57,3%
Butembo	43	20	46,5%
Cut	66	45	68,2%
Me	30	20	66,7%
Oicha	3	2	66,7%
Kalunguta	2		50,0%
Kyondo	5	1	60,0%
Ten		3	0,0%
Masereka	1	0	0,0%
Vuh			100,0%
Be anxious.		0	0,0%
The museums	3	1	22,2%
SOUTH KIVU	1193	021	33,3%
Miti-Murhesa	3	1	33,3%
TOTAL	1830	648	35,4%

Over the past 24 hours, **38 new confirmed cases and 23 deaths (73.9% community) have been recorded** in the provinces of Ituri (32 cases and 18 deaths) and North Kivu (6 cases and 5 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,830 confirmed cases and 648 deaths, expressing an overall case fatality rate of **35.4%**.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.1%** of cases (n=1,663) and **85.3%** of deaths (n=553), representing a crude case fatality rate of **33.3%**, making it the epicenter of the epidemic. The health zones of Bunia (495 cases), Rwampara (373 cases), Mongbwalu (327 cases), Nizi (107 cases), and Nyankunde (97 cases) remain the most affected, accounting for approximately 84.1% of reported cases at the provincial level and 76.4% at the national level. In Nizi, the epidemic

is currently progressing faster than in other areas.

Severity of the epidemic in North Kivu

A total of **164 confirmed cases and 94 deaths** have been reported in North Kivu province since the start of the outbreak, representing a **crude case fatality rate of 57.3%**. This high case fatality rate is concentrated in the health zones of **Katwa** (68.2%), **Beni** (66.7%), and **Butembo** (46.5%), while the other health zones account for only 36.0% of the case fatality rate. The three health zones with the highest number of reported cases are Katwa (n=66), followed by Butembo (n=43) and Beni (n=30). A case fatality rate of 100% was observed in the exceptionally small Vuhovi health zone.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Meeting with His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health, members of the national coordination team, and technical and financial partners. Discussions focused primarily on the progress of the response and the situation in the provinces of Tshopo and Haut-Uélé.
- Commitment from local and provincial authorities in Bunia to support response teams in the fight against Ebola virus disease, following advocacy efforts led by the INSP and its partners

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 9, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		38	81	0	119
New Alerts		492	731	9	1232
Total alerts for the day		530	812	9	1351
Alerts investigated		329	731	9	1069
Investigation rate		62,1%	90,0%	100,0% 2	79,1%
Alerts confirmed	Alive	157	44	0	203
	Deceased	59	22		81
Total suspected cases for today		216	66	2	284

During the day of July 9, 2026, a total of 1,351 alerts were recorded in the provinces of Ituri and North Kivu, including 1,232 new alerts and 119 alerts reported from the previous day, demonstrating the maintenance of active epidemiological surveillance in the affected provinces.

The majority of alerts were reported in North Kivu, with 812 alerts, or 60.1%, compared to 530 alerts, or 39.2%, in Ituri and 0.6% in South Kivu.

Of the alerts recorded, 1,069 were investigated, representing an overall investigation rate of 79.1%. The investigation rate was 90.0% in North Kivu and 62.1% in Ituri, highlighting the need to strengthen investigative capacities in the latter province.

The investigations led to the confirmation of 284 suspected cases, including 203 living individuals (71.5%) and 81 deaths (28.5%). Ituri recorded the highest number of confirmed suspected cases, with 216 (76.1%), compared to 66 cases, or 23.2%, in North Kivu and 0.7% in South Kivu.

Table 4. Contact tracing of confirmed cases (1) as of July 9, 2026

Province	Contacts under follow-up (n)	Contacts vus (n)	Follow-up rate (%)
Ituri	10 744	8 136	75,7%
North Kivu	2 202	1 970	89,5%
South Kivu*	0	0	THAT
Total	12 946	10 106	78,1%

* End of follow-up for all contacts listed in the province of South Kivu.

As of July 9th, **12,946** contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, **10,106** were actually seen, corresponding to an overall follow-up rate of **78.1%**.

Ituri had the highest number of contacts under follow-up, 10,744 (82.9% of the total), with 8,136 contacts seen, representing a follow-up rate of 75.7%. In North Kivu, 2,202 contacts were under follow-up, of which 1,970 (89.5%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 9, 2026

Province	Number of new contacts		Contact names		Number of contacts	
	n	%	had become suspects	n	made in the last 21 days	%
Ituri	524	81,2%	4	66,7%	92	56,4%
North Kivu	121	18,8%	2	33,3%	71	43,6%
Total	645	100,0%	6	100,0%	163	100,0%

The day ended on July 9th with **645 new contacts** listed, the majority in Ituri (524, 81.2%), while North Kivu listed 121 (18.8%). Among the contacts monitored, six developed symptoms consistent with the definition of a suspected case, four in Ituri (66.7%) and two in North Kivu (33.3%). These contacts require immediate investigation and management in accordance with current procedures.

In addition, 163 contacts completed their 21-day follow-up period without developing symptoms consistent with the disease, including 92 in Ituri, or 56.4%, and 71 in North Kivu, or 43.6%.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 9, 2026

INDICATORS	Ituri	North Kivu	Global
Proportion of PoC enabled and PoE enhanced	100% (57/57)	100,0% (14/14)	100,0% (71/71)
Number of people who went through PoE/PoC testing, % of	99 663	56 282	155 945
travelers screened, % of travelers	93,2%	97,9%	94,9%
who washed their hands, % of travelers made	92,4%	97,9%	94,4%
aware of the issue	99,4%	97,9%	98,9%
Number of alerts notified	10	0	10
Number of validated live alerts	5	0	5
Number of lifeless bodies intercepted today	2	0	2
Number of problematic contacts intercepted today; % of validated live		0	0
alerts referred to CT/CTE; % of deceased bodies swabed	0 100,0%	ND	100,0%
	100,0%	ND	100,0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 104 samples collected and analyzed (**positivity 30.8%; n=32**).
- **North Kivu** : : 108 samples collected and analyzed (**positivity 5.6%; n=6**).
- **South Kivu** : 2 samples collected and currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: Forty-four (44) confirmed cases were addressed through ringside activities. Nine (9) households were identified, of which six (6) were decontaminated (66.7%). Three (3) healthcare facilities were identified, of which two (2) were decontaminated. In addition, 12 IPC/WASH assessments, 9 IPC/EDS kits or supplies were distributed, 18 community health workers were monitored and supported, one community health worker was decontaminated, and 7 vehicles were decontaminated. 12 public health workers, 26 members of the EDS teams, and 120 service providers were trained or briefed.

North Kivu: An IPC scorecard assessment was conducted in seven (7) health facilities in the health zones of Beni (3), Kibua (1), Musienene (2), and Oicha (1), with an average score of 35%. Twenty-two (22) healthcare providers were briefed on IPC, CREC, and community surveillance, and fifteen (15) providers from La Vérité Hospital were trained on IPC/WASH. In total, 337 providers and 16 PPLs were briefed during formative supervision sessions. Three (3) health zones, 17 places of residence in Butembo, and 16 households of cases were decontaminated. Twenty-nine (29) death alerts were reported, 27 bodies were swabs and secured, and two (2) sample collection failures were recorded.

4.5 Holistic care**Ituri:**

- By the end of the day on July 9th, 1,045 hot meals had been served: 141 to suspected cases, 162 to Confirmed cases: 708 in PPLs and 34 in accompanying persons.

Patient movement:

- 89 admissions were recorded in the CTE/CT/CI facilities of the three provinces, including 43 in Ituri, 44 in North Kivu and 2 in South Kivu, according to the latest available data. Discharges totaled 62 patients

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 9, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	540	188	27	755
Total admissions (24h)	43	44	2	89
Sorties (24h)				
Deceased (Suspects/Confessions)	10	0	0	10
Non-case	27	6	8	41
Healed	4	1	0	5
Escapees (Suspects/Confessions)	4	0	0	4
Transferred to the HGR	2	0	0	2
Total sorties	47	7	8	62
Patients in isolation (End of Day), including	536	225	19	780
confirmed cases (NC+AC) and	218	20	0	238
suspected cases	318	205	19	542
Overall occupancy rate	89,9%	160,0%	42,2%	100,3%

In total, **780** patients were in isolation, including 238 confirmed cases and 542 suspected cases, for an overall bed occupancy rate of **100.3%**.

Table 8. Key indicators of mental health and psychosocial support activities as of July 9, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	7 (38,9%)	3(100,0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	45 (30,4%)	4(100,0%)	0
New suspected cases that benefited from SMSPS interventions	8 (28,8%)	9(100,0%)	2
Suspected cases under follow-up that have received interventions SMSPS	42 (33,9%)	3(100,0%)	2 (100,0%)
Laboratory results have been announced.	27	4	1
Including positive ones	0	3	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	7
Number of accompanying persons at the CTE who benefited from interventions SMSPS	128	1	0
Number of PPLs who benefited from SMSPS interventions	19	1	0
Number of household and community members who benefited from SMSPS interventions	23	0	0

4.6 Continuity of care

- In total, **112 PPLs** are confirmed, of which **36 have recovered**, 35 have died (31.2%), and 21 are hospitalized in the province of Ituri.

4.7 Risk communication and community engagement

Ituri: 46 people were engaged during three advocacy meetings (39 men and 7 women); 3,355 people were reached during home visits (1,488 men and 1,867 women); 7,625 people were reached during educational talks and focus groups (3,689 men and 3,926 women); 13,250 people were reached during 45 mass awareness campaigns (5,750 men and 7,500 women); and 1,814 people were reached in churches, mosques and markets.

Community feedback: Five (5) pieces of feedback and informational messages were reported, of which four (4) were captured and one (1) was clarified, representing 25.0%. The main concerns related to care in Ebola Treatment Centers (ETCs), the involvement of recovered individuals in awareness campaigns, payment for COVID-19 treatment (RECO), and the response times of decontamination teams.

Support for the pillars of the response

Surveillance: 51 community alerts were reported, including 31 live alerts and 20 deaths, in the health zones of Bunia and Rwampara.

CREC: Seventeen (17) members of the Lita Health Zone management team (14 men and 3 women) were trained in community-based communication and monitoring. Six radio programs and six public service announcements were produced and broadcast, 36 community radio stations were engaged, and 15 posters and IEC materials were distributed.

North Kivu: Community mobilization and outreach: 379 community actors reported their activities; 11,119 households were visited; 25,038 people were reached with prevention messages during door-to-door visits; 1,140 people were reached through dialogues and educational talks; and 3,122 people were sensitized in public places. Thirty-four (34) radio stations broadcast at least one media segment on Ebola Virus Disease (EVD).

Community surveillance and feedback management: 139 alerts of suspected cases were reported by community actors; six (6) feedback or infodemics were captured and resolved within 48 hours; and 1,135 people were engaged in community actions. In the Vuhesi and Mutendero health districts of the Vuhovi health zone, the CREC supported infection prevention and control (IPC) and surveillance by strengthening the capacity of healthcare personnel in the triage and isolation of suspected cases.

Specific feedback: In the Mutwanga Health Zone, religious leaders from Kasindi-Lubiriha reported insufficient IPC supplies and swabs to respond to community death alerts. In the Beni Health Zone, community actors in the Mandrale Health Association requested greater involvement and regular compensation for local community health workers in awareness-raising activities.

4.8. Logistics

Ituri

- Handover of two (2) motorcycles, donated by the Government, to the Nyankunde Health Zone and deployment of two (2) vehicles for the EDS teams in the Lita Health Zone. In total, 25 vehicles and five (5) ambulances have

has been made available to the various pillars to ensure the transfer of cases and the mobility of teams in the field.

North Kivu

- IPC supplies for the EDS teams were delivered, including hand sanitizer, body bags for children and adults, Tyvek covers, protective eyewear, PPE, PPL kits, disinfectant, and soap. The logistics teams participated in a briefing on the management of humanitarian and medical supplies, continued monitoring the repair of Mobile 103 in Mutsora, and held the section meeting. Four (4) out of 11 Health Zones submitted their logistics data; 50 motorcycles and 6 vehicles were reported as operational, while no ambulances or hearses were reported as operational in the tracking table.

4.9. Security

- Supervision was conducted at the Kasitu PoC to reinforce the deployed personnel's understanding of Ebola prevention measures and the role of screening. At the Kitatumba Ebola Treatment Center, the PNC personnel assigned to the site were briefed on the strict adherence to barrier measures. No new major security incidents were reported.
- Continued strike by service providers in the health zones of Bunia and Rwampara in Ituri.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Community awareness-raising activities on the prevention of sexual exploitation and abuse continued in the health zones of Bunia, Rwampara, Mongbwalu, and Aru. A total of 107 people were reached (54 women/girls and 53 men/boys). The cumulative total of briefings and signing of commitment agreements reached 2,188 people (1,241 men and 947 women), while the cumulative total of awareness-raising sessions reached 12,872 people (6,944 men and 5,928 women).

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2 Insufficient capacity of CTE/CT facilities and saturation in North Kivu (160.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3 Contact tracking performance not optimal (78.1% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet ventilated by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct data reconciliation, remove potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (112 cases) HCW (including 35 deaths, case fatality rate 31.2%)	Urgent supply, strengthening of IPC training

8	Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (57.3%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromise	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES

National Coordination Meeting





**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

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**Avec l'appui technique de l'Organisation Mondiale
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